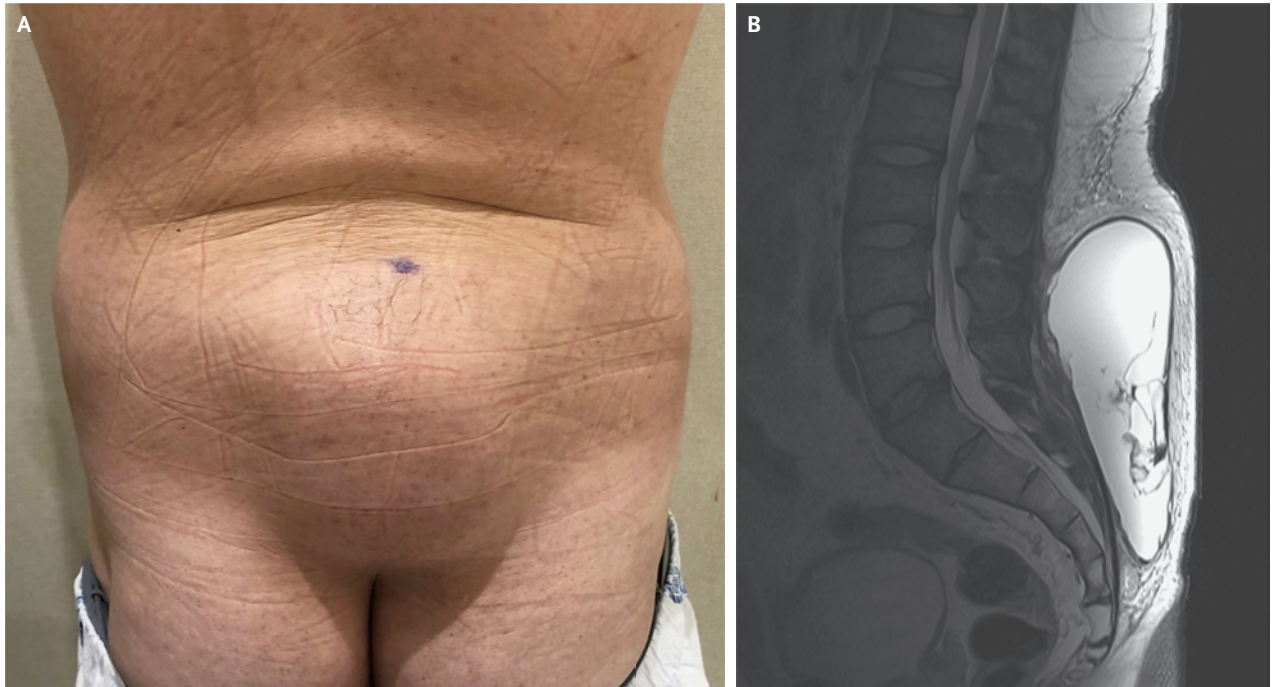


## IMAGES IN CLINICAL MEDICINE

Stephanie V. Sherman, M.D., *Editor*

## Morel–Lavallée Lesion



A HEALTHY 36-YEAR-OLD MAN PRESENTED TO THE ORTHOPEDIC CLINIC with an enlarging lower back mass. He had first noticed the mass 3 weeks earlier, after he slipped on the stairs and slid down on his back. On physical examination, a large, soft, fluctuant mass with no overlying skin necrosis was palpated on the lower back (Panel A). Magnetic resonance imaging of the lumbo-sacral spine showed an encapsulated, subcutaneous, suprafascial lesion, measuring 20 cm by 20 cm by 10 cm, that extended from the lower lumbar area to the buttocks (Panel B), a finding consistent with a Morel–Lavallée lesion. A Morel–Lavallée lesion is a traumatic, closed degloving injury that results from shearing of subcutaneous tissue away from the underlying fascia. Its management varies; in this case, surgical excision was performed because of the risk of recurrence, given its encapsulation and chronicity. Histologic examination of the excised cystic mass showed a pseudocyst lined with myofibroblasts. Three months after the surgery, the patient had recovered fully without recurrence of the mass.

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